

****DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Patient Name: Patient 07

Age: 77 years old

Sex: Male

MRN: 000-TEST-07

Admit Date: [admission date]

Discharge Date: [discharge date]

Hospital: Metro Medical Center

PRIMARY DIAGNOSIS & HOSPITAL COURSE

Patient 07 presents with acute decompensated congestive heart failure (CHF), systolic type, with reduced ejection fraction (EF 28% on transthoracic echocardiogram performed [date]). BNP level at admission was 847 pg/mL, consistent with moderate-to-severe ventricular dysfunction. Patient initially presented with dyspnea on exertion (DOE), orthopnea, and lower extremity edema. Chest X-ray revealed bilateral pleural effusions with pulmonary edema.

****Hospital Events & Interventions:****

- Admitted to telemetry unit for continuous cardiac monitoring
- IV furosemide initiated for diuresis; net negative 4.2 liters over 3-day hospitalization
- Echocardiography confirmed dilated LV with global hypokinesis
- Daily weights documented; baseline admission weight 89 kg
- Troponin I and BNP levels trended appropriately with diuretic therapy
- Patient diuresed to euvolemia; crackles resolved, orthopnea improved
- Discharged on optimized oral diuretic regimen with stable renal function (Cr 1.3)

****SECONDARY DIAGNOSES:**** Anemia (Hgb 10.1 g/dL at discharge, iron studies pending outpatient workup); chronic lower back pain; hypertension; type 2 diabetes mellitus (well-controlled on current regimen).

****DISCHARGE MEDICATIONS****

| Medication | Dose | Frequency | Duration |

|---|---|---|---|

| Lisinopril (ACEI) | 20 mg | q.d. (once daily) | Ongoing |

| Carvedilol (beta-blocker) | 12.5 mg | BID | Ongoing |

| Furosemide (loop diuretic) | 40 mg | BID | Ongoing; reassess in 2 weeks |

| Spironolactone (aldosterone antagonist) | 25 mg | q.d. | Ongoing |

| Metformin | 500 mg | BID with meals | Ongoing |

| Acetaminophen | 650 mg | Q6H PRN | For chronic pain |

| Omeprazole | 20 mg | q.d. | Ongoing |

****ACTIVITY & WEIGHT-BEARING STATUS****

- Ambulate as tolerated; no heavy lifting (>10 lbs) for 2 weeks
- Gradual increase in activity; patient counseled on DOE limits
- Continue cardiac rehabilitation referral (outpatient)

****DIETARY RESTRICTIONS****

- Strict 2-gram sodium diet; patient/family education provided
- Fluid restriction: 1.5 liters daily
- Avoid NSAIDs (ACEI/diuretic combination increases hyperkalemia/renal dysfunction risk)

****DAILY WEIGHT MONITORING****

Patient instructed to weigh self each morning before breakfast. Report weight gain ≥ 3 pounds in 24 hours or ≥ 5 pounds in 1 week to primary care physician or cardiologist immediately.

****FOLLOW-UP APPOINTMENTS****

1. Cardiology clinic: 2 weeks post-discharge (for EF reassessment, medication titration, BNP recheck)
2. Primary Care Physician: 1 week post-discharge (for vitals, renal function panel, K⁺ level)
3. Iron studies & hematology referral: